Topic 8 – Obstetrics, Gynaecology & Paediatrics

GPSC CBRT · Homoeopathic Physician (Advt 06/2026) · Oct 20, 2026 · @raghavendra

How to use this guide

Topic 8 gives about 2 of the 25 core marks. Questions are one-line facts: a number (weeks, days, mL, mg/dL), a definition, a milestone age, a vaccine schedule, or the homoeopathic remedy for a clear picture. Learn the tables and diagrams first.

Syllabus heading (Advt 06/2026)	What it covers	Section
Antenatal care and labour	Normal pregnancy, high-risk conditions (preeclampsia, gestational diabetes), stages of labour	8.1
Gynaecological pathologies	Dysmenorrhoea, abnormal uterine bleeding, PCOD, pelvic inflammatory disease, common pelvic neoplasms	8.2
Neonatology and child development	Developmental milestones, neonatal jaundice, infant feeding, malnutrition, rickets	8.3
Paediatric therapeutics and emergencies	Dentition disorders, respiratory infections, ADHD, enuresis, national child immunisation	8.4

Standard books. D. C. Dutta, *Textbook of Obstetrics*; D. C. Dutta, *Textbook of Gynecology*; *Shaw's Textbook of Gynaecology*; O. P. Ghai, *Essential Pediatrics*; K. Park, *Textbook of Preventive and Social Medicine* (immunisation). Homoeopathic therapeutics: Boericke's *Materia Medica with Repertory*; Allen's *Keynotes*; Lilienthal, *Homoeopathic Therapeutics*; Dewey, *Practical Homoeopathic Therapeutics*; Guernsey, *Application of the Principles and Practice of Homoeopathy to Obstetrics*. Vaccine schedules follow the Universal Immunization Programme (UIP), Ministry of Health and Family Welfare.

Plan to 1 November. One section a day for four days, then the rapid-revision sheet and the MCQs; after that, ten minutes a day on the rapid-revision sheet.

8.1 Antenatal care and labour

Most questions here are numbers: pregnancy length, number of visits, blood-pressure and glucose cut-offs, and the start and end of each stage of labour.

Normal pregnancy

Item	Fact
Duration	280 days (40 weeks) from the first day of the last menstrual period (LMP); 266 days from ovulation
Naegele's rule	Expected date of delivery = LMP + 9 months + 7 days (or LMP − 3 months + 7 days + 1 year)
Term	37–42 completed weeks; preterm before 37; post-term after 42
Trimesters	First up to 12 weeks, second 13–28, third 29–40
Earliest sign	Missed period; urine β-hCG positive about 10–14 days after conception (peaks at 8–10 weeks)
Quickening	First fetal movements felt at 18–20 weeks (primigravida), 16–18 weeks (multigravida)
Fetal heart	Heard on Doppler at 10–12 weeks; seen on ultrasound at 6 weeks; normal rate 110–160/min
Fundal height	Pubic symphysis at 12 weeks, umbilicus at 24 weeks, xiphisternum at 36 weeks
Weight gain	About 11–12 kg over the pregnancy
Signs	Hegar's sign (softening of the lower uterine segment, 6–10 weeks); Chadwick's sign (bluish vagina and cervix); Goodell's sign (soft cervix)

Antenatal care in India. At least **4 antenatal visits** (first in the first trimester, ideally before 12 weeks); the WHO now advises 8 contacts. Each visit: weight, blood pressure, urine for protein and sugar, haemoglobin, abdominal examination. **Iron and folic acid**: 1 tablet daily (60 mg elemental iron + 500 µg folic acid) for at least 180 days from the second trimester, continued 180 days after delivery; **folic acid 400 µg from before conception** prevents neural tube defects. **Calcium** 1 g daily from 14 weeks. **Tetanus–diphtheria (Td)** vaccine: 2 doses 4 weeks apart in the first pregnancy (Td-1 early, Td-2 four weeks later), or 1 booster if vaccinated in the previous 3 years.

Hook: "4 visits, 180 days, 2 doses" — four antenatal visits, 180 days of iron, two Td doses.

High-risk conditions

Condition	Definition	Key facts
Gestational hypertension	BP 140/90 mmHg or more after 20 weeks, no proteinuria	Returns to normal within 12 weeks after delivery
Preeclampsia	Hypertension after 20 weeks **with proteinuria** (300 mg or more in 24 h) or other organ involvement	Severe: BP 160/110 or more, headache, visual disturbance, epigastric pain, low platelets; HELLP syndrome (haemolysis, elevated liver enzymes, low platelets)
Eclampsia	Preeclampsia + **convulsions**	An obstetric emergency; delivery is the definitive cure
Gestational diabetes (GDM)	Glucose intolerance first recognised in pregnancy	India (DIPSI, single step): 75 g oral glucose, **2-hour value 140 mg/dL or more** = GDM, regardless of fasting state; screen at the first visit and again at 24–28 weeks
Anaemia in pregnancy	Hb below 11 g/dL (WHO)	Severe below 7 g/dL; commonest cause is iron deficiency
Antepartum haemorrhage	Bleeding after 28 weeks (India)	**Placenta praevia**: painless, causeless, recurrent bright red bleeding. **Abruptio placentae**: painful bleeding, tense tender uterus
Ectopic pregnancy	Implantation outside the uterine cavity	Commonest site: **ampulla of the fallopian tube**; triad: amenorrhoea, abdominal pain, vaginal bleeding
Hyperemesis gravidarum	Severe vomiting with dehydration, weight loss, ketosis	Commoner in multiple pregnancy and molar pregnancy

Condition	Definition	Key facts
Rh isoimmunisation	Rh-negative mother, Rh-positive fetus	Haemolytic disease of the newborn; anti-D given to Rh-negative unsensitised mothers

Complications of GDM for the baby. Macrosomia (birth weight over 4 kg), shoulder dystocia, neonatal hypoglycaemia, respiratory distress.

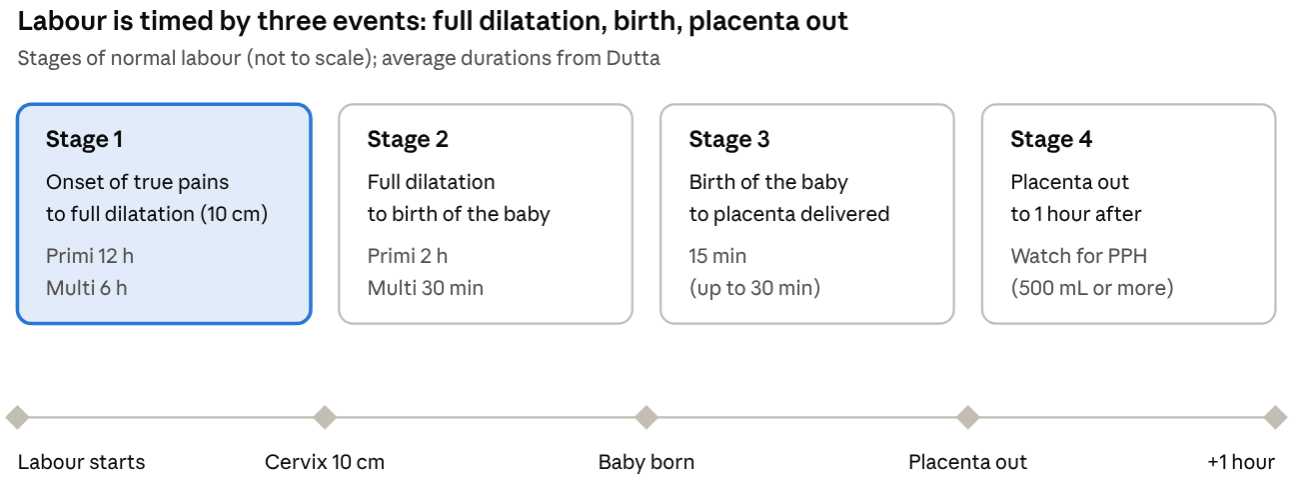
Stages of labour

Stage	From	To	Average duration
First	Onset of true labour pains	**Full dilatation of the cervix (10 cm)**	Primigravida 12 h; multipara 6 h
Second	Full dilatation	**Delivery of the baby**	Primigravida 2 h (1 h before epidural era); multipara 30 min
Third	Delivery of the baby	**Delivery of the placenta and membranes**	15 min (up to 30 min)
Fourth	Delivery of the placenta	1 hour after (observation)	1 h

The first stage has a **latent phase** (up to 4 cm, slow) and an **active phase** (from 4–5 cm, about 1 cm per hour in a primigravida). **True labour pains** are regular, increase in frequency and strength, cause cervical dilatation and are not relieved by sedation; false pains are irregular and do not dilate the cervix.

Cardinal movements of the fetus (vertex): Engagement, Descent, Flexion, Internal rotation, Extension, Restitution, External rotation, Expulsion. **Hook: "Every Decent Fetus Is Exceptionally Restless, Externally Expelled."**

Postpartum haemorrhage (PPH). Blood loss of 500 mL or more after vaginal delivery (1,000 mL after caesarean). Commonest cause: **uterine atony**. Causes: the "4 Ts" — Tone, Trauma, Tissue (retained placenta), Thrombin (clotting). **Puerperium** lasts 6 weeks; lochia: rubra (days 1–4), serosa (5–9), alba (10–15).



Stages of labour · the event that ends each stage and its duration

Each stage ends with one event on the line below it: remember the events and you remember the stages. Stage 1 (highlighted) is the longest.

Homoeopathic therapeutics in pregnancy and labour

Clinical picture	Remedy
Morning sickness: constant nausea not relieved by vomiting, clean tongue	Ipecacuanha
Nausea from the smell or thought of food, with indifference to the family and bearing-down	Sepia
Nausea and vomiting better from eating, worse in the morning, with irritability	Nux vomica
False labour pains, spasmodic, flying about; rigid cervix in a nervous woman	Caulophyllum, Gelsemium
Labour pains with weeping, changeable mood, wants fresh air; malposition	Pulsatilla
Labour with intolerable pain, angry, "cannot bear it"	Chamomilla
After-pains and bruised soreness after delivery	Arnica
Bright red, profuse bleeding with nausea	Ipecacuanha

In eclampsia, antepartum haemorrhage and PPH, the first duty is emergency obstetric care and referral; remedies support, never replace, it.

8.2 Gynaecological pathologies

Know each condition's definition, its classic triad or criteria, and one or two remedies with a clear picture.

Menstrual cycle in one line

Cycle 28 ± 7 days; flow 2–7 days; blood loss about 35 mL (over 80 mL = heavy). Follicular phase (FSH, oestrogen) → **ovulation on about day 14, 14 days before the next period, triggered by the LH surge** → luteal phase (**progesterone** from the corpus luteum keeps the endometrium secretory). Menarche about 12–13 years; menopause about 45–51 years (12 months without a period).

Dysmenorrhoea

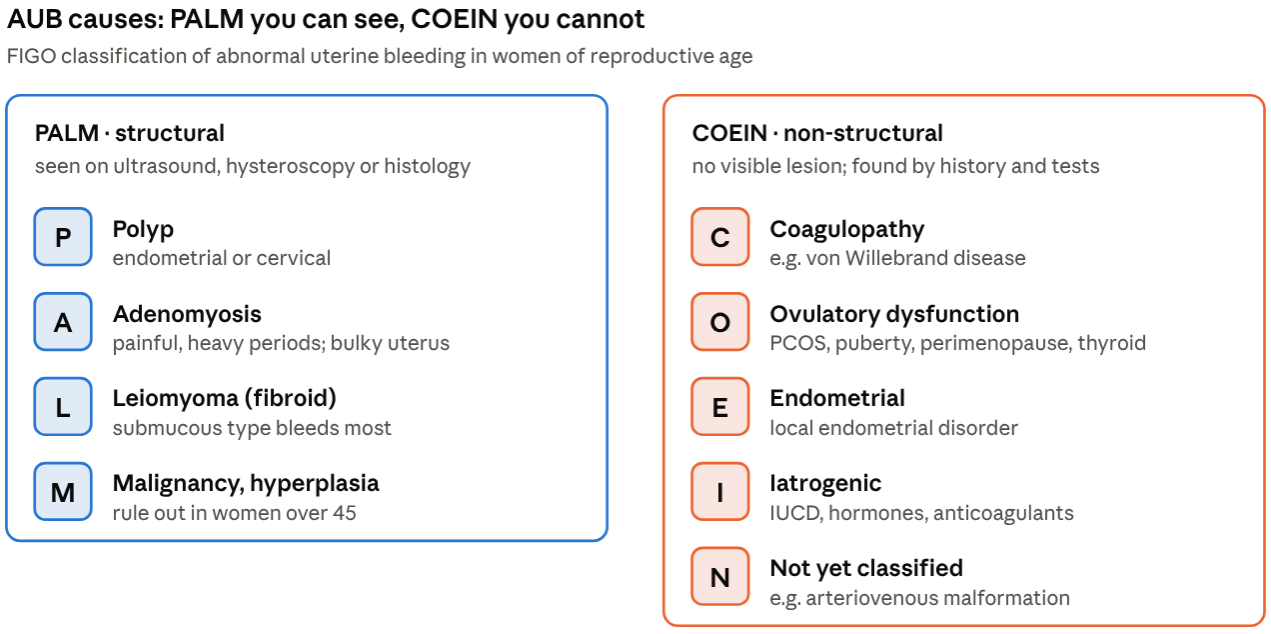
	Primary (spasmodic)	Secondary (congestive)
Cause	No pelvic disease; excess prostaglandins	Pelvic pathology: endometriosis, adenomyosis, PID, fibroid, IUCD
Onset	Within 6–12 months of menarche (once cycles become ovulatory)	Later in life, often after 30
Pain	Cramping, starts with the flow, lasts 12–72 h; lower abdomen and thighs	Dull, starts days before the flow, may continue after it
Course	Improves after childbirth and with age	Worsens with the disease

Abnormal uterine bleeding (AUB)

AUB is any bleeding abnormal in frequency, regularity, duration or volume. FIGO classifies its causes as **PALM-COEIN** (diagram below): the PALM causes are structural and seen on imaging or histology; the COEIN causes are not.

Old term	Meaning
Menorrhagia (heavy menstrual bleeding)	Regular cycles, heavy or prolonged flow (over 80 mL or over 7 days)
Metrorrhagia	Irregular bleeding between periods
Polymenorrhoea	Cycles shorter than 21 days
Oligomenorrhoea	Cycles longer than 35 days

Old term	Meaning
Amenorrhoea	Primary: no menarche by 15 years (with secondary sex characters) or by 13 (without them). Secondary: no period for 3 cycles or 6 months; **commonest cause is pregnancy**



PALM-COEIN · 4 structural and 5 non-structural causes of AUB

The commonest causes of AUB at puberty and perimenopause are ovulatory (O); in a woman over 45, malignancy (M) must be excluded first.

Polycystic ovarian disease (PCOD / PCOS)

- **Rotterdam criteria (2003): any 2 of 3.** (1) Oligo- or anovulation; (2) clinical or biochemical hyperandrogenism (hirsutism, acne, raised testosterone); (3) polycystic ovaries on ultrasound (12 or more follicles of 2–9 mm, or ovarian volume over 10 mL).
- Features: irregular periods, obesity, acanthosis nigricans (insulin resistance), infertility. **LH : FSH ratio often over 2 : 1.**
- Long-term risks: type 2 diabetes, endometrial hyperplasia and cancer, cardiovascular disease.
- First-line management is lifestyle: a 5–10% weight loss often restores ovulation.

Pelvic inflammatory disease (PID)

Ascending infection of the upper genital tract (endometritis, salpingitis, tubo-ovarian abscess), mostly **Chlamydia trachomatis and Neisseria gonorrhoeae**. Signs: lower abdominal pain, fever, purulent discharge, **cervical motion tenderness**, adnexal tenderness. Complications: **infertility, ectopic pregnancy**, chronic pelvic pain, Fitz-Hugh–Curtis syndrome (perihepatitis).

Common pelvic neoplasms

Tumour	Key facts
Leiomyoma (fibroid)	Commonest benign tumour of the uterus (and of women); oestrogen-dependent, shrinks after menopause. Types: intramural (commonest), subserous, submucous (bleeds most). Heavy menstrual bleeding, pressure symptoms, infertility. Red degeneration in pregnancy; sarcomatous change under 0.5%
Adenomyosis	Endometrium within the myometrium; uniformly enlarged, tender uterus; painful, heavy periods in multiparous women over 35
Endometriosis	Endometrium outside the uterus, commonest on the ovary ("chocolate cyst"); dysmenorrhoea, dyspareunia, infertility
Ovarian tumours	Commonest benign: serous cystadenoma; commonest in young women: dermoid (mature cystic teratoma); **CA-125** marker for epithelial ovarian cancer
Carcinoma cervix	Commonest genital cancer in Indian women; caused by **HPV 16 and 18**; screening by Pap smear and VIA (visual inspection with acetic acid); HPV vaccine prevents it
Endometrial carcinoma	Postmenopausal bleeding; risk: obesity, nulliparity, diabetes, unopposed oestrogen

Hook: "Fibroid = Fat uterus that Floods; Endometriosis = Elsewhere."

Homoeopathic therapeutics

Clinical picture	Remedy
Cramping dysmenorrhoea, better bending double and from hard pressure	Colocynthis, Magnesia phosphorica (better from warmth)
Bearing-down sensation as if everything would come out, must cross the legs; indifference	Sepia
Scanty, late, changeable periods in a mild, weepy, thirstless girl who wants consolation	Pulsatilla
Profuse, bright red flow with gushes, worse from motion	Sabina
Heavy, dark, clotted menses; pain from back to pubis	Sabina, Trillium
Sudden profuse bright bleeding with nausea	Ipecacuanha
Fibroids with heavy bleeding in a chilly, fat woman who sweats on the head	Calcarea carbonica
Right-sided ovarian pain, hot, throbbing	Belladonna (acute), Lycopodium (chronic)
Left-sided ovarian pain, worse after sleep, cannot bear tight clothing	Lachesis

8.3 Neonatology and child development

Milestone ages and growth numbers are the most-asked facts in paediatrics; neonatal jaundice is asked as "physiological or pathological".

The newborn

Item	Normal
Birth weight	2.5–4 kg (India average about 2.8 kg); **low birth weight under 2.5 kg**
Length / head circumference	50 cm / 34–35 cm
Heart rate / respiratory rate	120–160 / 40–60 per minute

Item	Normal
APGAR score	At **1 and 5 minutes**: Appearance (colour), Pulse, Grimace (reflex), Activity (tone), Respiration; each 0–2; 7–10 normal
Weight changes	Loses up to 10% in the first week, regains birth weight by **10–14 days**; doubles by **5 months**, triples by **1 year**, quadruples by **2 years**
Fontanelles	**Posterior closes by 2–3 months; anterior by 12–18 months**
First milk tooth	Lower central incisor at about **6 months**; all 20 milk teeth by 2–2½ years; first permanent tooth (first molar) at 6 years
Primitive reflexes	Moro, rooting, sucking, grasp: present at birth, disappear by 3–6 months

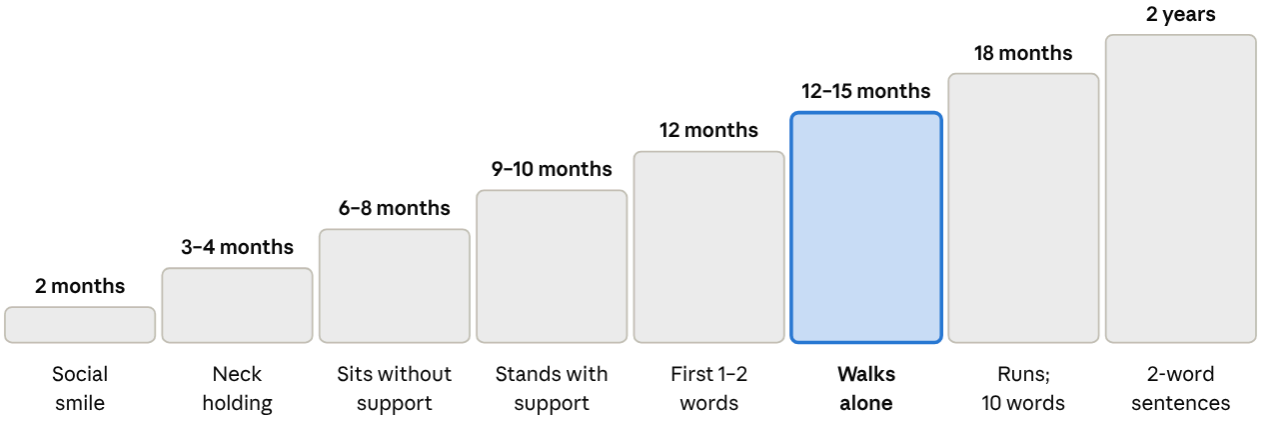
Kangaroo mother care: skin-to-skin contact and exclusive breastfeeding for low-birth-weight and preterm babies.

Developmental milestones

The ladder below shows the key milestone ages. **Red flags**: no social smile by 3 months, no head control by 4 months, not sitting by 9 months, not walking by 18 months, no words by 18 months.

Milestones build on each other: smile, neck, sit, stand, talk, walk

Usual ages from Ghai (steps not to scale); walking alone is highlighted as the most-asked milestone



Developmental milestones · 8 key steps from 2 months to 2 years

Climb the staircase left to right: each skill needs the one before it, so the order is as important as the age.

Age	Gross motor	Fine motor	Language	Social
2 months	Lifts head in prone	Follows objects	Coos	**Social smile**
3–4 months	**Neck holding**	Hands open, reaches	Laughs aloud	Recognises mother
6 months	Sits with support; rolls over	Transfers objects hand to hand	Monosyllables (ba, da)	Stranger anxiety begins
8–9 months	**Sits without support**; crawls	Pincer grasp begins (9 months)	Bisyllables (mama, dada)	Waves bye-bye
12 months	**Walks with support / stands alone**	Mature pincer grasp	**1–2 meaningful words**	Plays simple games
15 months	**Walks alone**	Tower of 2 cubes; scribbles	Jargon	Points to wants
18 months	Runs, climbs stairs with help	Tower of 3–4 cubes	10 words	Copies parents
2 years	Up and down stairs, 2 feet per step	Tower of 6–7 cubes	**2-word sentences**	Parallel play
3 years	Rides tricycle; climbs stairs one foot per step	Copies a circle; tower of 9	Sentences, knows age and sex	Shares toys
4–5 years	Hops, skips	Copies a square (4), triangle (5)	Tells a story	Cooperative play

Hook: "Smile 2, Neck 3, Sit 6–8, Stand 9–10, Walk 12–15, Words 12, Phrases 24."

Neonatal jaundice

	Physiological	Pathological
Appears	**After 24 hours** (day 2–3)	**Within 24 hours** of birth (always pathological)

	Physiological	Pathological
Peak	Day 3–5 (term)	Any time; rises fast (over 5 mg/dL per day)
Lasts	Disappears by **14 days** (term), 21 days (preterm)	Beyond 2 weeks (term)
Bilirubin	Unconjugated, under about 15 mg/dL	Conjugated over 2 mg/dL is always pathological
Causes	Immature liver conjugation, short RBC life	Rh or ABO incompatibility, sepsis, G6PD deficiency, biliary atresia, hypothyroidism

Very high unconjugated bilirubin can cross into the brain: **kernicterus** (bilirubin encephalopathy). Treated with phototherapy (blue light) and, if severe, exchange transfusion. Breast-milk jaundice appears after the first week and is harmless.

Infant feeding

- **Breastfeeding within 1 hour of birth**; colostrum (first 3–4 days) is rich in protein, IgA and vitamin A.
- **Exclusive breastfeeding for 6 months**, then complementary feeding from **6 months**, continuing breastfeeding to **2 years or beyond**.
- Breast milk has more lactose and less protein than cow's milk; whey : casein about 60 : 40; protective factors: secretory IgA, lactoferrin, lysozyme, bifidus factor.
- Energy of breast milk about 67 kcal/100 mL; a baby needs about 150 mL/kg/day.

Malnutrition and rickets

Condition	Key facts
Severe acute malnutrition (6–59 months)	**MUAC under 11.5 cm**, or weight-for-height under −3 SD, or bilateral pitting oedema
Kwashiorkor	Protein deficiency with adequate calories: **oedema**, flaky-paint dermatosis, sparse pale hair (flag sign), fatty liver, apathy; weight 60–80% of expected
Marasmus	Deficiency of total calories: **severe wasting, no oedema**, "old man" face, alert and hungry; weight under 60%

Condition	Key facts
Rickets	Vitamin D deficiency before the growth plates fuse: bowed legs, **rachitic rosary** (costochondral beading), Harrison's sulcus, craniotabes, delayed fontanelle closure, widened wrists; raised **alkaline phosphatase**, low phosphate
Osteomalacia	Vitamin D deficiency after the growth plates fuse (adults)
Vitamin A deficiency	Night blindness first, then Bitot's spots, keratomalacia; dose under the national programme: 9 doses from 9 months to 5 years

Hook: "Kwashiorkor = Kwater (oedema); Marasmus = Muscle wasting."

Homoeopathic therapeutics. Marasmus with emaciation from the neck downward, ravenous appetite yet thin: **Natrum muriaticum**; emaciation beginning in the legs: **Abrotanum**. Rickets with large head, open fontanelles, sour sweat on the head and slow teething in a fat, flabby child: **Calcarea carbonica**; in a thin child craving smoked meat: **Calcarea phosphorica**; with offensive foot sweat and weak ankles: **Silicea**.

8.4 Paediatric therapeutics and emergencies

The vaccine schedule is the single most-asked item in this section; the rest is definitions plus a remedy picture.

Dentition disorders

Teething begins at about 6 months with drooling, irritability and gum rubbing. Fever above 38.5 °C or diarrhoea should not be blamed on teething without looking for another cause.

Picture	Remedy
Painful teething, one cheek red and hot, the other pale; angry, wants to be carried; green, hot, offensive stools	**Chamomilla**
Slow, delayed dentition; late-closing fontanelles; thin child craving smoked meat	**Calcarea phosphorica**
Delayed teething in a fat, flabby child with sour sweat on the head	**Calcarea carbonica**

Picture	Remedy
Teeth decay as soon as they appear; black teeth	**Kreosotum**
Teething with convulsions, hot head, cold limbs, dilated pupils	Belladonna

Respiratory infections

WHO/IMNCI fast-breathing cut-offs (count for a full minute): **60 or more** under 2 months, **50 or more** from 2 to 12 months, **40 or more** from 1 to 5 years. Chest indrawing or any general danger sign (unable to drink, vomits everything, convulsions, lethargy) means severe illness: refer urgently.

Picture	Remedy
Sudden high fever after exposure to dry cold wind, anxious, restless, thirsty (first stage)	**Aconitum napellus**
High fever with flushed face, throbbing, dilated pupils, no thirst	**Belladonna**
Coarse rattling of mucus in the chest, but cannot cough it up; drowsy	**Antimonium tartaricum**
Croup: dry, barking, "sawing" cough, better from warm food and drink	**Spongia tosta**
Croup with loose rattling and choking, worse from cold air, very sensitive	**Hepar sulphuris**
Whooping cough: spasmodic, barking, worse after midnight, with nosebleed	**Drosera**
Bland coryza and cough, thick yellow-green discharge, wants open air	Pulsatilla

Behavioural problems

ADHD (attention deficit hyperactivity disorder). Inattention, hyperactivity and impulsivity before the age of 12, in two or more settings (home and school), for at least 6 months; commoner in boys. Remedies for restless, destructive, hurried children: **Tarentula hispanica** (restless, destructive, music ameliorates), **Stramonium** (fears, violence, night terrors), **Hyoscyamus** (jealous, silly, exposing), **Tuberculinum** (wants change, destructive).

Enuresis. Involuntary wetting **after the age of 5 years** (developmental bladder control). Primary (never dry) is commonest and often familial; secondary (dry for 6 months, then wetting) suggests stress, infection or diabetes. Remedies: **Equisetum** (enuresis with dreams of urinating), **Causticum** (in the first sleep, also on coughing), **Kreosotum** (deep sleep, hard to wake), **Sepia** (in the first sleep, girls).

Other. Breath-holding spells (6 months to 5 years, harmless); night terrors; **encopresis** (soiling after age 4); pica (eating non-food items, look for iron deficiency).

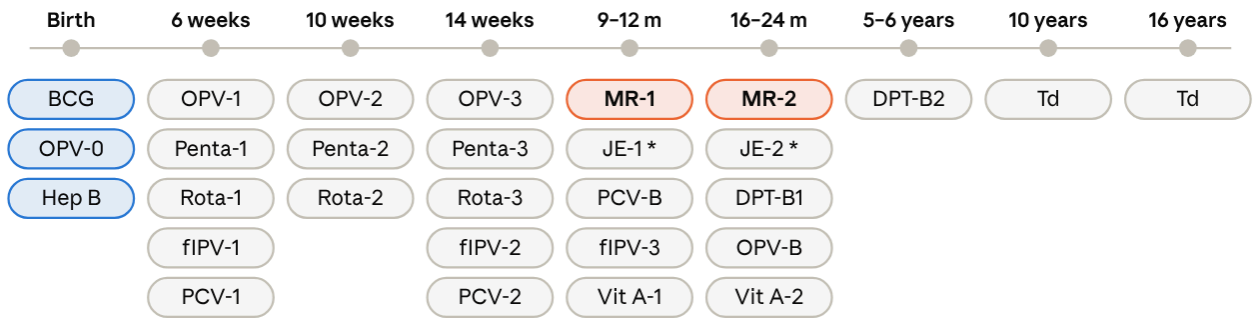
National immunisation (Universal Immunization Programme)

Age	Vaccines
Birth	**BCG**, **OPV-0**, **Hepatitis B birth dose** (within 24 h)
6 weeks	OPV-1, **Pentavalent-1** (DPT + Hep B + Hib), Rotavirus-1, **fIPV-1**, PCV-1
10 weeks	OPV-2, Pentavalent-2, Rotavirus-2
14 weeks	OPV-3, Pentavalent-3, Rotavirus-3, fIPV-2, PCV-2
9–12 months	**MR-1** (measles–rubella), JE-1 (endemic districts), PCV booster, fIPV-3, **Vitamin A (1st dose)**
16–24 months	**MR-2**, JE-2, **DPT booster-1**, OPV booster, Vitamin A (2nd dose; then every 6 months to 5 years)
5–6 years	**DPT booster-2**
10 years and 16 years	**Td**
Pregnant women	Td-1, Td-2 (or booster)

Facts asked. BCG is given intradermally on the left upper arm, up to 1 year of age. OPV is the only oral vaccine besides rotavirus. Measles vaccine is live and given at 9 months because maternal antibodies interfere earlier. Mission Indradhanush targets children and pregnant women missed by routine immunisation; Pulse Polio gives OPV to all children under 5. India was declared polio-free in 2014. **Hook: "Birth = BOH (BCG, OPV, Hep B); 6-10-14 weeks = POR (Penta, OPV, Rota); 9 months = MR."**

Most vaccines are given in the first year: birth, 6, 10, 14 weeks, 9 months

Universal Immunization Programme, India (not to scale)



Penta = DPT + Hepatitis B + Hib. fIPV = fractional injectable polio vaccine. PCV-B = pneumococcal booster. * JE: endemic areas. Vitamin A: 1st dose at 9 months, then every 6 months to 5 years (9 doses). Pregnant women: Td-1 and Td-2 four weeks apart. Blue = birth doses; orange = measles-rubella (MR), the most-asked schedule item.

National immunisation schedule · 9 visits from birth to 16 years

Read the chart column by column: the three primary doses at 6, 10 and 14 weeks are identical except that PCV and fIPV skip 10 weeks.

Rapid revision: numbers and one-line facts

Area	Fact
Pregnancy	280 days from LMP; EDD = LMP + 9 months + 7 days; term 37–42 weeks
Pregnancy	Quickening 18–20 weeks (primi); fundus at umbilicus 24 weeks; FHR 110–160/min
Antenatal	4 visits minimum; IFA 180 days; Td 2 doses; folic acid 400 µg before conception
High risk	Preeclampsia = BP 140/90 after 20 weeks + proteinuria; eclampsia = + convulsions
High risk	GDM (DIPSI) = 2-hour glucose 140 mg/dL or more after 75 g; anaemia in pregnancy Hb under 11
High risk	Painless bleeding = placenta praevia; painful = abruptio; ectopic commonest in the ampulla
Labour	1st stage ends at full dilatation (10 cm); 2nd at delivery of the baby; 3rd at delivery of the placenta
Labour	PPH = 500 mL or more (vaginal); commonest cause atony; puerperium 6 weeks

Area	Fact
Gynaecology	Ovulation 14 days before the next period (LH surge); progesterone keeps the luteal phase
Gynaecology	Primary dysmenorrhoea: no pelvic disease, starts with the flow; secondary: endometriosis, PID, fibroid
Gynaecology	AUB = PALM-COEIN; PCOS = Rotterdam 2 of 3; PID causes infertility and ectopic pregnancy
Gynaecology	Commonest uterine tumour = fibroid; cervical cancer = HPV 16, 18; CA-125 = ovarian cancer
Newborn	LBW under 2.5 kg; APGAR at 1 and 5 min; regains birth weight by 10–14 days
Growth	Weight doubles 5 months, triples 1 year; anterior fontanelle closes 12–18 months
Milestones	Smile 2 m; neck 3–4 m; sit 6–8 m; stand 9–10 m; walk 12–15 m; 2-word phrases 2 years
Jaundice	Within 24 hours = pathological; physiological after 24 h, gone by 14 days
Feeding	Breastfeed within 1 hour; exclusive 6 months; continue to 2 years
Malnutrition	SAM = MUAC under 11.5 cm; kwashiorkor = oedema; marasmus = wasting
Rickets	Rosary, bowed legs, wide wrists; ALP raised
Paediatrics	Enuresis after 5 years; ADHD onset before 12 years, 6 months, two settings
Breathing	Fast breathing: 60 (under 2 m), 50 (2–12 m), 40 (1–5 y) per minute
Vaccines	Birth BCG, OPV-0, Hep B; 6-10-14 weeks Penta, OPV, Rota (+fIPV, PCV); 9 m MR-1; 16–24 m MR-2, DPT-B1; 5 y DPT-B2; 10 and 16 y Td
Remedies	Teething = Chamomilla; slow dentition = Calc phos; croup = Spongia; rattling = Ant tart; enuresis in first sleep = Causticum

Area	Fact
Remedies	Dysmenorrhoea bending double = Colocynth; bearing down = Sepia; bright red gushing = Sabina

Practice MCQs

Twenty questions grouped by syllabus heading; the answer key with a one-line reason follows.

Antenatal care and labour

1. If the LMP was 10 January, the expected date of delivery is about: (a) 3 October (b) 17 October (c) 10 October (d) 24 October
2. Preeclampsia is hypertension after 20 weeks with: (a) glycosuria (b) proteinuria or organ involvement (c) polyhydramnios (d) anaemia
3. In India, gestational diabetes is diagnosed (DIPSI) when the 2-hour glucose after 75 g is: (a) 120 mg/dL or more (b) 140 mg/dL or more (c) 180 mg/dL or more (d) 200 mg/dL or more
4. The second stage of labour ends with: (a) full dilatation of the cervix (b) delivery of the baby (c) delivery of the placenta (d) onset of pains
5. Painless, recurrent, bright red bleeding in the third trimester suggests: (a) abruptio placentae (b) placenta praevia (c) ruptured uterus (d) cervical polyp

Gynaecology

6. Primary dysmenorrhoea is defined as painful menstruation: (a) due to endometriosis (b) without pelvic pathology (c) only after childbirth (d) due to fibroid
7. In the PALM-COEIN classification, "L" stands for: (a) luteal phase defect (b) leiomyoma (c) lymphoma (d) lichen sclerosus
8. Polycystic ovarian syndrome is diagnosed by the Rotterdam criteria when present are: (a) all 3 criteria (b) any 2 of 3 (c) any 1 of 3 (d) raised LH only
9. The most important long-term complication of pelvic inflammatory disease is: (a) menorrhagia (b) infertility (c) fibroid (d) PCOS
10. Bearing-down sensation with indifference to family and better from vigorous exercise suggests: (a) Pulsatilla (b) Sepia (c) Sabina (d) Belladonna

Neonatology and development

11. A child usually sits without support at: (a) 3 months (b) 6–8 months (c) 12 months (d) 15 months

12. Jaundice appearing on the first day of life is: (a) physiological (b) pathological (c) breast-milk jaundice (d) normal in term babies
13. The anterior fontanelle closes by: (a) 2–3 months (b) 6 months (c) 12–18 months (d) 3 years
14. Oedema with flaky-paint skin and a fatty liver in a malnourished child indicates: (a) marasmus (b) kwashiorkor (c) rickets (d) scurvy
15. Exclusive breastfeeding is recommended for the first: (a) 2 months (b) 4 months (c) 6 months (d) 12 months

Paediatric therapeutics and immunisation

16. Which vaccines are given at birth under the UIP? (a) BCG, OPV-0, Hepatitis B (b) DPT, OPV, Hib (c) MR, Vitamin A (d) Rotavirus, PCV
17. The first dose of measles–rubella vaccine is given at: (a) birth (b) 6 weeks (c) 9–12 months (d) 5 years
18. Enuresis is diagnosed only after the age of: (a) 2 years (b) 3 years (c) 5 years (d) 10 years
19. Painful dentition in an angry child who wants to be carried, with one cheek red: (a) Calcarea phos (b) Chamomilla (c) Belladonna (d) Kreosotum
20. Croup with dry, barking, sawing cough better from warm drinks: (a) Spongia tosta (b) Antimonium tart (c) Drosera (d) Pulsatilla

Answer key

Q	Answer	Why
1	b	10 Jan + 9 months + 7 days = 17 October
2	b	Proteinuria (300 mg/24 h) or end-organ involvement
3	b	DIPSI single-step 75 g test, 2-hour value 140 mg/dL
4	b	2nd stage = full dilatation to delivery of the baby
5	b	Praevia is painless; abruptio is painful
6	b	No pelvic pathology; prostaglandin-mediated
7	b	PALM = polyp, adenomyosis, leiomyoma, malignancy
8	b	Any 2 of anovulation, hyperandrogenism, polycystic ovaries

Q	Answer	Why
9	b	Tubal damage: infertility and ectopic pregnancy
10	b	Sepia keynotes
11	b	Sits without support 6–8 months
12	b	Jaundice within 24 hours is always pathological
13	c	Anterior 12–18 months; posterior 2–3 months
14	b	Oedema = kwashiorkor
15	c	6 months, then complementary feeding
16	a	BCG, OPV-0, Hep B birth dose
17	c	MR-1 at 9–12 months, MR-2 at 16–24 months
18	c	Bladder control is expected by 5 years
19	b	Chamomilla keynote
20	a	Spongia: dry sawing croup better warm drinks